

Your Autistic Teenager

For parents & family carers — supporting an autistic teenager (roughly ages 12–18).

The one idea

Autism, through **monotropism**, means your teen’s attention locks into a few deep, intense “tunnels.” Adolescence piles high-channel demands — shifting peer rules, dating, body changes, exams — on a system that finds rapid switching costly. This is the **peak age for anxiety, depression and burnout**, and many autistic teens (especially girls and marginalised genders) are **first identified only in crisis**. Their intense interests are identity, regulation and recovery — not problems. Your job shifts from managing to *accompanying*: follow their lead, protect their focus, lower the load, and take their inner life seriously.

What you may see — and what’s really going on

You may see...	What’s often happening
Exhausted, withdrawn, “can’t be bothered”; refusing school	Probable autistic burnout or overwhelm — distinct from depression and laziness
Intense interests; resists giving them up	Interests are identity, regulation and recovery — not obsessions
“Fine” all day, collapses behind a closed door	Heavy camouflaging ; the cost is paid later, at home
Self-harm, dark thoughts, or crisis	Real, elevated risk in this group; often the first sign of unmet need
Slow to start/switch; won’t answer questions	Autistic inertia + processing time, not rudeness or “not caring”

Try this

- **Protect and validate their interests** — never limit a healthy passion; it’s their main regulator and a route to study, friends and recovery.
- **Make space for unmasking at home** — home is the safe place where the school-day mask finally drops. Expect an after-school crash; build in calm, low-demand, interest-led time.
- **Lower demands when the tank is low**. If they’re burning out, the evidence-backed move is to **reduce expectations and restore rest and interests** — not push harder.
- **Communicate literally, one thing at a time**, often in writing or by text; give processing time; respect their autonomy in decisions.
- **Ask directly** about mood, anxiety, sleep and suicidal thoughts using concrete questions — and listen to the answer.
- **Advocate at school** (SENCO/SEND, EHCP/IEP, a quiet space, fewer transitions) and plan the move to adult services early.

Avoid this

- Confusing **autistic burnout with depression** — treating it as “try harder” can deepen it and raise suicide risk.
- Pressuring them to mask, make eye contact, or “be more social” as a goal.

- Piling on activities, social plans and expectations when they're already depleted.

When to get more support

Autistic burnout is real, serious and distinct — chronic exhaustion, loss of skills and reduced tolerance to stimulus; it can last months and is linked to suicidal thoughts. Co-occurring **anxiety, depression, ADHD (AuDHD), sleep and eating difficulties** are common and deserve autism-informed care. Seek an autism-positive clinician, peer groups and mentors — high-quality friendship is powerfully protective.

If your teen is a girl or marginalised gender

Camouflaging peaks here and drives much missed diagnosis and mental-health cost. The “anxious, perfectionist, socially-adept-on-the-surface” teen may be an exhausted autistic young person. Ask specifically about **masking exhaustion** and take a lifelong pattern of difference seriously.

About this guide

*AI-assisted, derived from this project's research drafts — the **Family guide** (Parts 1 & 4) and **Monotropism** brief, with the **Lifespan Practitioner & Health-Care guide** (Part 3.3). Uses identity-first language. **Informational — not medical advice or a diagnostic tool**. You know your teen best; fit this to them.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020, burnout); Pearson & Rose (2021, masking); Bradley et al. (2021, camouflaging); Edgar (2024). Full citations are in the source drafts.